

Laryngectomy

The main aim of laryngectomy surgery is to remove the cancer completely. You may have a partial laryngectomy or total laryngectomy.

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What is a laryngectomy?

Surgery is one of the main treatments for [laryngeal cancer](#). The main aim of laryngectomy surgery is to remove the cancer completely.

A partial laryngectomy is surgery to remove the part of the [larynx](#) affected by cancer. This operation is rare, as people are usually offered [radiotherapy](#) or [chemoradiation](#) instead.

Some people have an operation to remove the cancer by removing the whole larynx. This is called a total laryngectomy. Your doctors may offer this treatment if they think radiotherapy may not cure the cancer completely. You may also need some [lymph nodes](#) removed from your neck (see below).

Before a laryngectomy

Before you have the operation, your surgeon will explain what will happen before, during and after the surgery. You will have some tests to make sure you are well enough for the operation. These usually include:

- [blood tests](#)
- a chest [x-ray](#)
- an electrocardiogram (ECG) to check your heart
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breathing tests (lung function tests).

You may have these done at a pre-assessment clinic before you go into hospital.

You have this operation under general anaesthetic.

Partial laryngectomy

You might have a partial laryngectomy to treat [early stage cancer of the larynx](#) or if you need more treatment. Your surgeon will remove the affected part of the [larynx](#).

This may be done endoscopically. This means the surgeon passes the instrument down the throat. Or it may be done through a cut in the neck.

After a partial laryngectomy, you still have part of your larynx. You may have some difficulties with your voice or swallowing. This will depend on the type of partial laryngectomy you have.

Your surgeon and speech and language therapist (SLT) will discuss your specific operation, how it might affect you and ways to cope.

Tracheostomy

Sometimes, surgery or radiotherapy to the larynx causes temporary swelling around the throat. This can narrow your airway and make it difficult to breathe.

If this happens, the surgeon creates a small opening at the front of the neck. This allows them to insert a tube into the windpipe (trachea) to help you breathe. This is called a tracheostomy or stoma.

The tracheostomy is held open by a small plastic tube that is a few centimetres long. The tube stays in place until the swelling improves. This usually takes a few days. After this, the tube is removed. You breathe through your nose and mouth like you did before. When the tube is removed, the opening is left to heal naturally.

If your surgeon thinks you need a tracheostomy after surgery, they will talk to you about it beforehand. You will have time to ask the specialist nurse or SLT questions before you have surgery.

We have more information about having a [temporary tracheostomy](#).

Total laryngectomy

A total laryngectomy is an operation to remove all of the [larynx \(voicebox\)](#). You might need an operation to remove the whole larynx in order to remove all the cancer.

Your doctors may offer this treatment if they think [radiotherapy](#) may not cure the cancer completely. You may also need some [lymph nodes removed from your neck](#).

When the surgeon removes the whole larynx, there is no longer a connection between your mouth, nose and lungs. This means that after the operation you breathe and speak in a different way.

You breathe through a hole (opening) in your neck, called a [tracheostomy](#) or stoma. You also no longer have vocal cords to produce a voice. But you can speak using a choice of different ways to communicate. We have more information on [speaking after a laryngectomy](#).

The connection between the throat and oesophagus (gullet) is usually not affected. This means you can swallow liquids and food as you did before the operation. Sometimes you may need to eat softer foods to begin with.

Laryngectomy stoma

A laryngectomy stoma is when the surgeon makes a permanent opening in the windpipe. It is in the lower part of your neck and you breathe through it.

Having a stoma is safe. It usually stays open on its own and will not close over. Some people may need a soft tube to help keep their stoma open for the first few days after surgery. Others may need the tube for longer.

Image: Laryngectomy stoma

The thought of having a laryngectomy stoma can be frightening. But you will get lots of support and information from your [healthcare team](#).

You have the stoma for the rest of your life and start breathing through it when you have your operation. Your surgeon, specialist nurse and speech and language therapist (SLT) will talk to you about this before your operation.

When you are well enough, your nurse and SLT will teach you how to clean and take care of the stoma. You can continue to get support from your healthcare team for as long as you need it.

We have more information on [living with a laryngectomy stoma](#).

Removing lymph nodes

Surgery to remove the cervical [lymph nodes](#) in one or both sides of the neck is called a [neck dissection](#).

It tells your doctor more about the [stage of the cancer](#), removes any lymph nodes that contain cancer cells and reduces the risk of the cancer coming back.

A neck dissection is done under general anaesthetic. It can be done:

- at the same time as the operation to remove the cancer
- before or after [chemoradiation](#) or [radiotherapy](#) if the lymph nodes still have cancer cells in them
- if the cancer comes back in the nodes after treatment.

You have tests first to see if there are any signs of cancer in the lymph nodes. The nodes that are removed are sent to a laboratory. A pathologist checks them for cancer cells. Your surgeon will talk to you about this before your operation.

After the operation, your neck and shoulder may be stiff on the side where you had the surgery. A physiotherapist can show you exercises to help with this.

We have more information about what to expect [after your operation](#).

Reconstruction

Sometimes, you may need surgery to reconstruct the area where you have had the operation. The aim of reconstructive surgery is to make the area look and work as naturally as possible.

Your surgeon takes tissue from another part of the body such as the thigh, arm, or chest. They use it to replace tissue taken from the neck. This is known as a myocutaneous flap. Myo means muscle, and cutaneous means skin.

Reconstruction can help with your recovery from the operation. It can also help improve your speech and [swallowing](#). Your surgeon will explain whether this is recommended for you.

Finding support

You may find it useful to speak to other people who have had a laryngectomy, or other surgery which has changed the way they look. These support groups may be helpful:

- **Cancer Laryngectomy Trust**

The [Cancer Laryngectomy Trust](#) provides support to people who are about to have, or have had a laryngectomy.

- **Changing Faces**

[Changing Faces](#) offers advice and information to anyone who is affected by a change in their appearance.

- **National Association of Laryngectomy Clubs**

The [National Association of Laryngectomy Clubs](#) aims to promote the welfare of people who have had a laryngectomy, and their families.

About our information

References



Below is a sample of the sources used in our laryngeal cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

ESMO Annals of Oncology. Squamous cell carcinoma of the oral cavity, larynx, oropharynx and hypopharynx. EHNS-ESMO-ESTRO Clinical Practice Guidelines for diagnosis, treatment and follow up. 2020. Available from <https://www.esmo.org/guidelines/ehns-esmo-estro-clinical-practice-guideline-squamous-cell-carcinoma-of-the-head-and-neck> (accessed Jan 2022).

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Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Claire Paterson](#), Consultant Clinical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

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ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Head and neck cancer forum

An anonymous network of people affected by head and neck cancer which is free to join. Share experiences, ask questions and talk to people who understand.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help